

Artwork Document Information		Artwork Type [Please Tick /]		Sign / Stamp	
Product : L PROMEDYL DM LINCTUS 90ML [FRONT] SAP No. : Date : 04 September 2025		☐ Unit Box ☐ Outer Box ☐ Label ☒ Leaflet ☐ Fix-A-Form ☐ Shipper Carton ☐ Aluminium Foil ☐ Sachet ☐ Others : _____			
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PROMEDYL^{DM}

LINCTUS

SOOTHES DRY, IRRITATING COUGH

with nasal congestion

Sugar-Free • Colourant-Free • Alcohol-Free

DESCRIPTION
A flavoured, clear, colourless to straw coloured syrup

COMPOSITION
Each 5ml contains
Pseudoephedrine HCL 30mg
Dextromethorphan HBr 15mg
Chlorpheniramine Maleate 2.0mg
Contains preservatives
0.21% w/v Methylparaben & 0.031% w/v Propylparaben

PHARMACODYNAMICS
Pseudoephedrine is a direct- and indirect- acting sympathomimetic. It is a stereoisomer of ephedrine and has a similar action, but has been stated to have less pressor activity and fewer CNS effects. Pseudoephedrine salts are given by mouth for symptomatic relief of nasal congestion.

Dextromethorphan hydrobromide is a cough suppressant used for the relief of non-productive cough; it has a central action on the cough centre in the medulla. It is also an antagonist of N-methyl-D-aspartate (NMDA) receptors. Although structurally related to morphine, dextromethorphan has no classical analgesic properties and little sedative activity.

Chlorpheniramine Maleate, an alkylamine derivative, is a sedating antihistamine that causes a moderate degree of sedation; it also has antimuscarinic activity.

PHARMACOKINETICS
Pseudoephedrine is readily absorbed from the gastrointestinal tract. It is largely excreted unchanged in the urine together with small amounts of its hepatic metabolite. It has a half life of about 5 to 8 hours, elimination is enhanced and half life accordingly shorter in acid urine. Small amounts are distributed into breast milk.

Dextromethorphan is rapidly absorbed from the gastrointestinal tract. It is metabolize in the liver and excreted in the urine as unchanged dextromethorphan and demethylated metabolites including dextrophan which has some cough suppressant activity.

Chlorpheniramine maleate is absorbed relatively slowly from the gastrointestinal tract, peak plasma concentration occurring at about 2.6 to 6 hours after administration by mouth. Bioavailability is low, values of 25 to 50% having been reported. Chlorpheniramine appears to undergo considerable first pass metabolism.

About 70% of chlorpheniramine in the circulation is bound to plasma proteins. There is wide interindividual variation in the pharmacokinetics of chlorpheniramine; Values ranging in from 2 to 43 hours have been reported in the half life.

Chlorpheniramine is widely distributed in the body, including passage into the CNS.

Chlorpheniramine maleate is extensively metabolised. Metabolites include desmethyl- and didesmethylchlorpheniramine. Unchanged drug and metabolites are excreted primarily in the urine; dependent on urinary pH and flow rate. Only trace amounts have been found in the faeces. A duration of action of 4 to 6 hours has been reported. More rapid and extensive absorption, faster clearance, and a shorter half life have been reported in children.

INDICATION
Promedyl DM is indicated for relief of dry irritating cough and nasal congestion.

SIDE EFFECTS
Common side effects
Sedation, varying from drowsiness to deep sleep. Tachycardia, hypertension, anxiety, and restlessness. Other effects include mild fatigue, dizziness, diarrhoea, constipation, nausea and vomiting.

Occasional side effects
Skin rashes and other hypersensitivity reactions. Urinary retention and other antimuscarinic effects like dry mouth, chest tightness and blurred vision.

Rare side effects
Hypotension, tinnitus, headache and paraesthesia. Hallucinations have been reported rarely particularly in children.

WARNING AND PRECAUTIONS
Sensitivity to any of the medication in the combination being taken. Patients sensitive to one antihistamines or sympathomimetics may be sensitive to others.

Use with caution for patients with Asthma, Bladder neck obstruction, Hepatic insufficiency, Narrow angle glaucoma, Pyloroduodenal obstruction, Stenosing peptic ulcer, Symptomatic prostatic hypertrophy, Arrhythmias, Diabetes mellitus, Hyperthyroidism, Ischemic Heart Disease and Renal failure.

Use is not recommended in newborn or premature infants because this age group has an increased susceptibility to anticholinergic side effects, such as central nervous system (CNS) excitation, and an increased tendency towards convulsions. A paradoxical reaction characterized by hyperexcitability may occur in children taking antihistamines.

When used for treatment of cough & cold
1. Not to be used in children less than 2 years of age
2. To be used with caution and doctor's/pharmacist's advice in children 2 to 6 years of age.

Use in elderly
Dizziness, sedation, confusion and hypotension may be more likely to occur in geriatrics patients. A paradoxical reaction characterised by hyperexcitability may occur. Geriatrics are more susceptible to the anticholinergic effects such as dry mouth and urinary retention (especially in males). If these side effects occur and continue or are severe, medications should probably be discontinued.

PREGNANCY AND LACTATION
Care must be taken for the fetus or newborn infant, especially with high dose and/or long term usage. Antihistamines may cause excitement and irritability in nursing infant. Pseudoephedrine causes high risks for infants due to effects from sympathomimetic amines.

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CONTRAINDICATION

Hypersensitivity to the ingredients. Contraindicated in severe coronary artery disease, severe hypertension and in patients receiving MAOI's therapy.

INTERACTIONS WITH OTHER MEDICAMENTS

Central Nervous system (CNS) depression-producing medication

Concurrent use may potentiate the CNS depressant effects of these medications, dextromethorphan or antihistamines. Concurrent use of maprotiline or tricyclic anti depressants may potentiate the anticholinergic effects of either these medications or antihistamines.

Monoamine oxidase (MAO) inhibitors including furazolidone and procarbazine

Concurrent use may prolong or identify the anticholinergic and CNS depressant effects of antihistamines; not recommended for concurrent use.

Anticholinergics or medications with anticholinergic effects

Anticholinergic effects may be potentiated when these medications are concurrently with antihistamines; patients should be advised to report occurrence of gastrointestinal problems promptly since paralytic ileus may occur with concurrent therapy.

RECOMMENDED DOSE

Adult

10 ml three times a day

Children

6 - 12 years: 5ml three times a day

2 - 5 years: 2.5 ml three times a day

Proper use of the medication

Medication should be taken with food, water or milk to minimize irritation.

Drink a glass of water after each dose of medication to help loosen mucus in lungs.

SYMPTOMS AND TREATMENT OF OVERDOSE

SYMPTOMS

Convulsions, fast breathing, hallucinations, increase in blood pressure, irregular heart beat, continuing, shortness of breath or troubled breathing; severe or continuing unusual nervousness, restlessness or excitement.

TREATMENT

Treatment is symptomatic and supportive

- a. Emetics and gastric lavage within 4 hours of overdosage.
- b. Charcoal is useful only if administered after one hour.
- c. Forced diuresis will increase elimination, provided adequate renal function but not recommended in severe overdosage.
- d. Cardiac state monitored and serum electrolytes measured.
- e. If there are signs of cardiotoxicity, intravenous propranolol may be indicated.
- f. For delirium and convulsions, intravenous diazepam may be administered.

STORAGE CONDITION

Keep container tightly closed.
Store in a dry place below 30°C.
Protect from light.
Keep out of reach of children.
Jauhi dari kanak-kanak.

Route of administration: Oral

Shelf life: Please refer to outer package.

PACKAGING / PACK SIZE

90ml
Not all pack sizes are available locally

Product Registration Holder:

Duopharma Marketing Sdn. Bhd.
Lot No. 2, 4, 6, 8 & 10, Jalan P/7, Section 13,
Bangi Industrial Estate, 43650 Bandar Baru Bangi,
Selangor, Malaysia.

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